Royal Devon University Healthcare NHS Foundation Trust

Clinical Guideline for:
Pregnancy Loss - Under 14 Weeks Gestation

Summary

This guideline includes information for nurses, midwives and the Bereavement Officer who are required to care for women experiencing early pregnancy loss. Guidance covers clinical care as well as support services/psychological care. This guideline is relevant to departments Trust-wide, e.g. the Emergency Department.

Key Points

The essential elements of this guideline are:

1. The importance of caring for the woman and her family in an empathetic and sensitive manner from diagnosis to discharge.
2. Clear guidance in the care of women experiencing early pregnancy loss
3. Clear guidance for completing the documentation following delivery.
4. The role of the Bereavement Officer and options for funeral services.

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Hyperlinks (to documents on the HUB Library)

Surgical TOP for Abnormality before 14+0 Care Pathway

National Bereavement Care Pathway

1. INTRODUCTION

1.1 Although it is recognised that pregnancy loss could occur within other departments of the Trust, i.e. the Emergency Department or ward areas, this document has been prepared primarily for staff in the Gynaecology Unit within Specialist Services Division. All Registered Nurses and associated professionals will adhere to this policy when caring for women and their families who experience early pregnancy loss.

1.2 Any member of staff caring for women and families who experience pregnancy loss and have any questions, or who may be unsure of any aspects of this policy, will need to contact the either the Lead Bereavement midwife (07881 039359), Wynard Ward Clinical Nurse Manager (01392 406512), Gynaecology Clinical Matron (01392 408362) or the Bereavement Office (01392 402349/6576).

1.3 This guideline has been developed to incorporate the Standards of the National Bereavement Care Pathway 2022.

2. AIM

To ensure that appropriate organisational arrangements are in place to secure acceptable standards of care and support for women and their families who have experienced pregnancy loss at less than 14-weeks gestation.

3. OBJECTIVES

To provide all relevant staff working within the RDUH with a framework to provide individualised care for women and families experiencing a pregnancy loss that is sensitive to cultural, religious and individual preferences.

4. RESPONSIBILITIES

4.1 It is the responsibility of the Clinical Matron Gynaecology and the Lead Bereavement Midwife to ensure all staff are trained and informed appropriately to provide both clinical and support care for women and their families.

4.2 Bereavement care training will be available via a national e-learning package and in-house training in conjunction with Women's Health Counsellors and Lead Bereavement Midwife.

4.3 It is the responsibility of the Clinical Matron to ensure all documentation is relevant and standard Trust-wide.

4.4 The Lead Bereavement Midwife will be responsible for liaising with appropriate Trust personnel and associated external providers. He/she will need to liaise with the Clinical Matron with regard to any suggested alterations to the contents of this policy/documentation.

5. EARLY PREGNANCY LOSS CARE

5.1 This section covers all women who experience the loss of their pregnancy by way of ectopic pregnancy, miscarriage or fetal abnormality.

5.2 Women need to be cared for in identified accommodation, ideally a single room where possible.

5.3 It is essential that staff provide information, both written and verbal. This needs to be clear and sensitive, explaining the full process of the woman's admission.

5.4 All communication with parents experiencing a pregnancy loss must be empathic, sensitive, non-judgemental and parent-led. Remember too that shock and distress can mean that information given is not absorbed and may need to be repeated or provided in writing (or other formats). Parents may have had different experiences of accessing medical care and of communicating with medical professionals. Some may find it less easy to ask questions, request clarification or express their wishes. Use warm, open body language by sitting near parents, facing them, making eye contact and using touch if appropriate. Be mindful of your tone and background noise if communicating over the telephone. Be honest with parents while being as sensitive as possible. It is okay to show emotion. An interpreter should be used when required. All efforts should be made to provide information in a chosen language and in easy read and digital formats.

5.5 Language used to describe a pregnancy loss must reflect the woman's preferences. For example, if a woman describes her pregnancy loss as a baby, this must be documented and used on every occasion the pregnancy loss is discussed.

5.6 Every effort should be made to involve the partner/family according to the woman's wishes and individual needs. A parent led approach should be adopted that enables women and their families to make informed choices about all aspects of their ongoing care. The woman can have someone present to support them at all times.

5.7 Informed consent should be sought at the earliest opportunity to send any products of conception (POC) for histology to confirm pregnancy and exclude molar change. POC are sent to histology in formalin and requested on EPIC. Following histology, POC are sensitively disposed of in a shared cremation, with consent.

5.8 The nurse must outcome and resolve the pregnancy episode on EPIC. This will remove the stork icon and prevent future correspondence to the patient regarding further midwifery appointments.5.9 A discharge summary must be sent to the GP and Midwife within 24 hours of the pregnancy loss to inform them of the patient's loss, this is sent electronically.

6. FUNERAL ARRANGEMENTS

6.1 Parents may not want the POC to be cremated by the hospital and may wish to arrange a funeral/burial of their own. This may be due to cultural, religious or spiritual beliefs and is a personal choice. Parents can arrange a funeral regardless of how early in the pregnancy the pregnancy loss occurred.

6.2 If the parents would still like to have histology and arrange their own funeral/burial arrangements, then part of the sample needs to be sent to histology in a container with formalin with the routine request on EPIC, and the rest of the POC can be placed in a container with saline (not formalin). This needs to be labelled and the parents can either take it home directly from the ward/clinic or it can be transferred to the mortuary for a funeral director to collect on the parent's behalf. ***Ensure the parents understand that the part of the pregnancy tissue which sent for histology is not possible to return.***

6.3 The doctor or nurse/midwife should complete a notice of death form, authorisation for burial or cremation form and taking deceased baby home by parents form. These forms should be uploaded to EPIC and a copy also sent to the bereavement officer. The bereavement officer will then be able to call the parents to discuss the options available to them and also to take verbal consent to pass their baby into the care of the chosen funeral director if it has been sent to the mortuary. ***Ensure the parents are given the bereavement officer details.***

7. HELPING PARENTS WITH BEREAVEMENT

7.1 There are a number of factors that can help parents and families through the bereavement process. Staff must respect and recognise that everyone may have individual needs or requests. On occasions there may be requests based on individual cultural/spiritual needs.

7.2 All bereaved parents are informed about, and if requested referred for emotional support and for specialist mental health support when needed, through Women's Health Counsellor or external services

7.3 During the woman's time in hospital they should be offered a Heartsease folder which contains relevant information to support them through the bereavement and counselling services should also be discussed and details given.

7.4 Remembrance Book – this is kept on Labour Ward. Families can make an inscription to remember their baby. A book of remembrance form needs to be completed (found in the heartsease pack) and sent to Labour Ward. Parents are able to come back to look at the book by appointment. See appendix A

7.5 Parents can place a ribbon on the tree of remembrance at the Centre for Women's Health - details of these can be discussed with the Bereavement Office.

7.6 All bereaved parents are offered opportunities to make memories. Memory boxes are located on Wynard Ward and Clinic 2 and can be taken home with the parents.

8. PSYCHOLOGICAL CARE

8.1 It is recognised that women/families may need psychological support. In some cases, this will need to be on-going. The Counselling Service can offer this support within the Unit, which can be provided whilst the women are inpatients and for four months following discharge.

8.2 The Department of Pastoral, Religious and Spiritual Care has close involvement with the Unit and can be contacted via switchboard if the parents would like spiritual support.

CWH Counsellor Email: rde-tr.cwhcounsellors@nhs.net Tel: 01392 406678

9. ROLE OF BEREAVEMENT OFFICER

9.1 The Bereavement Officer will be responsible for co-ordination of support services in relation to pregnancy.

9.2 The Bereavement Officer will support and direct families with funeral arrangements and documentary procedures as required.

9.3 The Bereavement Officer will be the nominated point of contact for external agencies.

9.4 The Bereavement Officer will work closely with clinical staff, liaising on administrative requirements.

9.5 The Bereavement Officer will liaise with all staff and ensure all documents and details are correct.

9.6 There will be no expectation of clinical duties with the Bereavement Officer role.

9.7 The Bereavement Officer will be an effective resource for clinical staff in relation to funeral service administration. He/she will be expected to maintain up-to-date knowledge.

Bereavement Officer contact: 01392 402349/6576
Mortuary general enquiries: 01392 403060
Mortuary Manager: 01392 403052

10. REFERENCES

10.1 *The NMC Code of Professional Conduct Standards of Conduct, Performance and Ethics* (Nursing and Midwifery Council, 2018).

10.2 *Handbook on Cultural, Spiritual and Religious Beliefs.* (RD&E Foundation NHS Trust).

10.3 *Patient Affairs*. (Bereavement Services).

10.4 *Miscarriage: Women's experiences and needs* (Christine Moulder *(*Routledge) 2001).

10.5 *Miscarriage, Ectopic Pregnancy and Molar Pregnancy (National Bereavement Care Pathway 2022)*

11. ASSOCIATED CLINICAL GUIDELINES OR POLICIES

Please refer to Hub for guidance relating to pregnancy loss at later gestation.

12. PUBLICATION DETAILS

Author of Clinical Guideline	Clinical Matron Gynaecology Lead Bereavement Midwife Mortuary Services
Directorate/Department responsible for Clinical Guideline	Specialist Services/CWH/Gynaecology
Contact details	Ext. 8362
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Appendix A Book of Remembrance Form

Royal Devon and Exeter NHS
NHS Foundation Trust

Bereavement Midwives
Labour Ward
Royal Devon & Exeter Hospital
Barrack Road
Exeter
EX2 5DW

BOOK OF REMEMBRANCE

A book of Remembrance is kept at this hospital in which parents can remember babies who died during pregnancy, shortly after delivery, or whilst receiving neonatal care. This is available for viewing on Labour Ward.

If you would like your baby to be remembered in this book, please complete this form below and write any words you wish to be included and return it to the above address.

If you would like to write a poem or anything else please attach it to this sheet. If you would like to do this in your own handwriting, and/or include a drawing, these can be done on paper up to A4 size.

Baby's Name (If named)	
Date of Birth	
Names (s) of Parent (s)	
Contact Telephone Number	

Words be entered into Remembrance book:

SAMPLE